

## CITY HEART INSTITUTE

Department of Cardiology | Patient Clinical Notes

Patient Name: John Smith Member ID: UHC-887612 Physician: Dr. Ramesh Kumar  
DOB: 03/15/1968 Gender: Male Policy #: UHC-POL-2026-001 NPI: 1234567890  
Insurance: United Healthcare Date: 03/01/2026 Specialty: Cardiology

Chief Complaint / Presenting Hx:

57 yr old ♂ w/ progressive dyspnea on exertion x3 months, bilateral ankle edema + orthopnea. No chest pain. PMHx: HTN, DM type 2. Meds: metoprolol, lisinopril, metformin.

Diagnosis:

Dilated Cardiomyopathy ICD-10: I42.0

Examination Findings:

BP: 148/92 mmHg HR: 88 bpm RR: 18/min SpO<sub>2</sub>: 96%  
Elevated JVP ++. S3 gallop heard at apex. Bilateral basal crepitations. Pitting pedal edema 2+. No murmur.

Investigations / Labs:

BNP: 450 pg/mL ↑ (nl <100)  
Metabolic Panel: WNL ✓  
ECG: Sinus rhythm, LVH pattern, non-specific ST changes.  
Echo (prelim): EF ~30-35%. Diffuse LV hypokinesis.

Procedure Ordered:

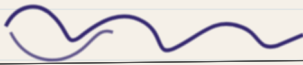
Cardiac MRI with Contrast CPT: 75563  
Facility: City Heart Institute — Cardiology Dept.  
Indications: assess myocardial viability, EF quantification, exclude ischemic etiology, fibrosis mapping (LGE).

Plan / Management:

- 1) Admit for monitoring + IV diuresis (Lasix 40mg BD)
- 2) Optimise GDMT — add spironolactone, uptitrate BB
- 3) Cardiac MRI as above (pre-auth UHC Gold Plus req'd)
- 4) Cardiology f/u in 2 wks post-MRI
- 5) Pt counselled re: sodium restriction, fluid balance, weight monitoring. ICD discussion deferred.

Allergies: NKDA

Group: GRP-7721 Plan: UHC Gold Plus

Physician Signature: 

Date signed: 01/03/2026 Dr. Ramesh Kumar, MD | Cardiology | NPI: 1234567890

